

1. Administrative & Study Identification

Full Study Title: Durvalumab ± Tremelimumab in Combination With Platinum Based Chemotherapy in Untreated Extensive-Stage Small Cell Lung Cancer (CASPIAN) **Official Title:** A Phase III, Randomized, Multicenter, Open-Label, Comparative Study to Determine the Efficacy of Durvalumab or Durvalumab and Tremelimumab in Combination With Platinum-Based Chemotherapy for the First-Line Treatment in Patients With Extensive Disease Small-Cell Lung Cancer (SCLC) (CASPIAN) **Short Title/Acronym:** CASPIAN **Protocol Number:** D419QC00001 **NCT Number:** NCT03043872 **Posting ID:** 995038253080036812 **Sponsor/Company Name:** AstraZeneca **Investigational Product:** Durvalumab (anti-PD-L1) and Tremelimumab (anti-CTLA-4) **Phase of Development:** PHASE3 **Trial Status:** ACTIVE_NOT_RECRUITING **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the efficacy of durvalumab + tremelimumab + platinum-etoposide (EP) treatment compared with EP alone, and the efficacy of durvalumab + EP treatment compared with EP alone, in terms of Overall Survival (OS). **Secondary Objectives:** To evaluate investigator-assessed Progression-Free Survival (PFS), Objective Response Rate (ORR), Duration of Response (DoR), and overall safety and tolerability.

2.2 Background & Rationale To address the poor prognosis and significant unmet medical need in patients with extensive-stage small-cell lung cancer (ES-SCLC). Most patients with SCLC have extensive-stage disease at presentation. Recently, immunotherapy has demonstrated clinical activity in ES-SCLC. This study aims to evaluate whether adding immune checkpoint inhibitors (PD-L1 inhibitor durvalumab, with or without CTLA-4 inhibitor tremelimumab) to standard first-line platinum-based chemotherapy provides a significant overall survival benefit.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Standard of Care Platinum-Etoposide [EP]) **Blinding:** Open-label, sponsor-blind **Randomization:** 1:1:1, stratified according to the planned platinum-based therapy for Cycle 1 (cisplatin or carboplatin).

3.2 Planned Enrollment & Duration Target **\$N\$:** 987 patients **Number of Sites:** 209 sites across 23 countries **Estimated Study Start:** 27-Mar-2017 **Estimated Primary Completion:** 27-Jan-2020 (Estimated Study Completion: 31-Dec-2025)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years with treatment-naïve, histologically or cytologically documented extensive-stage small cell lung cancer (American Joint Committee on Cancer Stage (7th edition) IV SCLC [T any, N any, M1 a/b], or T3-4 due to multiple lung nodules that are too extensive or have tumor/nodal volume that is too large to be encompassed in a tolerable radiation plan). 2. World Health Organization (WHO) / Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1 at enrolment. 3. Measurable disease according to Response Evaluation Criteria in Solid Tumors (RECIST) version 1.1. 4. Life expectancy ≥ 12 weeks at Day 1. 5. Suitable to receive a platinum-based chemotherapy regimen as 1st line treatment. 6. Brain metastases must be asymptomatic or treated and stable off steroids and anti-convulsants for at least 1 month prior to study treatment.

4.2 Exclusion Criteria 1. Any history of radiotherapy to the chest prior to systemic therapy or planned consolidation chest radiation therapy (except palliative care outside of the chest). 2. Prior exposure to immune-mediated therapy excluding therapeutic anticancer vaccines. 3. Paraneoplastic syndrome of autoimmune nature, requiring systemic treatment or clinical symptomatology suggesting worsening of PNS. 4. Active infection including tuberculosis, HIV, hepatitis B and C. 5. Active or prior documented autoimmune or inflammatory disorders. 6. Uncontrolled intercurrent illness, including but not limited to interstitial lung disease.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Arm 1:** Durvalumab 1500 mg + Tremelimumab 75 mg + EP every 3 weeks for 4 cycles, followed by one additional dose of tremelimumab and maintenance durvalumab 1500 mg every 4 weeks. * **Arm 2:** Durvalumab 1500 mg + EP every 3 weeks for 4 cycles, followed by maintenance durvalumab 1500 mg every 4 weeks. * **Arm 3:** Platinum-etoposide (EP) alone every 3 weeks for up to 6 cycles. (Note: Chemotherapy consists of etoposide 80-100 mg/m² on Days 1-3, and investigator's choice of carboplatin AUC 5-6 or cisplatin [ATC Code: L01XA01, Trade Name: Platinol] 75-80 mg/m² on Day 1 of each 21-day cycle). **Route of Administration:** Intravenous (IV) **Duration of Treatment:** Until objective disease progression or unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted: Prophylactic cranial irradiation (PCI) is permitted at the investigator's

discretion exclusively in the control arm (EP alone) if clinically indicated. **Prohibited:** PCI is not permitted in the experimental immunotherapy arms.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	Day -28 to 0	Informed Consent, Medical History, Baseline Tumor Assessment
2	Day 1 (Q3W/Q4W)	Randomization, IV Infusions (Durvalumab $\pm$ Tremelimumab + EP), Safety Labs
3	Week 6 $\pm$ 1 week	Tumor Assessment (RECIST v1.1), Safety Evaluation
4	Week 12 $\pm$ 1 week	Tumor Assessment (RECIST v1.1)
5	Every 8 weeks $\pm$ 1 week	Tumor Assessments until confirmed objective disease progression
6	Variable	Survival Follow-Up, Final Vital Signs, Exit Interview

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints

- Overall Survival (OS) in the Global Cohort; Assessed at Global Cohort Interim Analysis:** D + EP Compared With EP. Defined as time from randomization until death due to any cause. Assessed at approximately 60% maturity (318 OS events per group) with Data Cut-Off (DCO) 11 March 2019.
- OS in the Global Cohort; Assessed at Global Cohort Final Analysis:** D + EP Compared With EP and D + T + EP Compared With EP. Assessed at the global cohort final analysis with DCO 27 January 2020.
- OS in the China Cohort; Assessed at China Cohort First Analysis:** D + EP Compared With EP. Pre-specified at approximately 60% maturity with DCO 06 January 2020.

7.2 Secondary & Exploratory Endpoints

- OS in the Global Cohort:** D + T + EP Compared With D + EP.
- Progression-Free Survival (PFS):** In the Global Cohort.
- Objective Response Rate (ORR):** In the Global Cohort.
- Duration of Response (DoR).**
- Safety and tolerability.**

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of adverse events, assessed in all patients who received at least one dose of their assigned study treatment. **Serious Adverse Events (SAEs):** Standard SAE reporting within 24 hours. Focus on immune-related adverse events requiring monitoring and patient education. **Data Monitoring Committee (DMC):** An Independent Data Monitoring Committee (IDMC) comprised of independent experts was convened to confirm the safety and tolerability of the proposed dose and schedule of durvalumab $\pm$ tremelimumab in combination with platinum-based chemotherapy at two early stages of enrollment.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intention-to-Treat (ITT) population (all randomized patients) for efficacy outcomes; Safety Set (all patients who received at least one dose of study treatment) for safety outcomes. **Sample Size Justification:** Target N=987\$ appropriately powered to detect a clinically meaningful improvement in OS. Kaplan-Meier technique is utilized for calculating median OS.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study was conducted in accordance with the International Conference on Harmonization (ICH) Good Clinical Practice (GCP) guidelines, the Declaration of Helsinki, and applicable local regulations. **Monitoring Plan:** Regular onsite and remote monitoring visits, including review by the IDMC.

11. Study Identifiers & Governance

Primary ID: D419QC00001 **Registry Number:** NCT03043872 **Posting ID:** 995038253080036812 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** CASPIAN **Official Regulatory Title:** A Phase III, Randomized, Multicenter, Open-Label, Comparative Study to Determine the Efficacy of Durvalumab or Durvalumab and Tremelimumab in Combination With Platinum-Based Chemotherapy for the First-Line Treatment in Patients With Extensive Disease Small-Cell Lung Cancer (SCLC) (CASPIAN)

12. Clinical Framework (Oncology Specific)

Primary Condition: Extensive-Stage Small Cell Lung Cancer (ES-SCLC) **Disease Areas:** Small Cell Lung Carcinoma Extensive Disease **Preferred Name / UMLS Name:** Small cell lung cancer extensive stage / Small Cell Lung Carcinoma, Extensive Stage **Semantic Type:** Neoplastic Process **Definition:** Small cell lung carcinoma which has spread beyond one hemi-thorax and the regional lymph nodes. **MeSH Code:** D009369 **SNOMED ID:** 708064003 (Hierarchy: 188554009 | Extensive stage small cell lung carcinoma | Extensive stage small cell lung carcinoma (disorder)) **Diagnosis Threshold:** Histologically or cytologically documented extensive disease ES-SCLC **Medical Methodology:** First-line immune checkpoint blockade combined with cytotoxic chemotherapy **Optimization Target:** Improvement of Overall Survival (OS) and Progression-Free Survival (PFS)

13. Study Procedures & Methodology

Management Pathway: Standard platinum-etoposide clinical pathway augmented with PD-L1 $\pm$ CTLA-4 inhibitors. **Education Protocol (HCP):** Recognition and management of immune-related adverse events (irAEs) associated with durvalumab and tremelimumab. **Education Protocol (Patient):** Informed consent, reporting of immune-mediated symptoms, and treatment schedule adherence.

14. Observation & Follow-Up Schedule

Total Duration: From randomization until confirmed disease progression, unacceptable toxicity, withdrawal of consent, or study completion, followed up for survival. **Onsite Visit Cadence:** Every 3 weeks (Induction), then every 4 weeks (Maintenance). **Radiological Review Interval:** Week 6 $\pm$ 1 week, Week 12 $\pm$ 1 week, and then every 8 weeks $\pm$ 1 week.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]					